DTP Rx Aggregator

Content Strategy & Sitemap

Project Blueprint — March 2026

Your independent guide to direct-to-patient prescription drug programs
Aggregating, comparing, and simplifying DTP access for patients and HCPs

1. Executive Summary

This document outlines the complete content strategy and sitemap for a website that will serve as the first independent, clinically-informed aggregator of direct-to-patient (DTP) prescription drug programs in the United States.

Mission Statement

To bridge the trust gap between patients and direct-to-patient prescription drug programs by providing an independent, pharmacist-built platform that patients and healthcare providers can rely on — free from manufacturer influence, free from sales bias, and built entirely around the patient's best interest. We exist because patients deserve a neutral guide, not a pharma storefront.

The Problem We Solve

- Patients are deeply skeptical of manufacturer-run DTP platforms — when LillyDirect or PfizerForAll is the source, patients instinctively feel they are being sold to, not helped. This perception barrier is preventing millions of patients from accessing programs that could save them significant money
- TrumpRx.gov launched in February 2026 with 43 medications but functions primarily as a landing page, not a comparison tool
- AmericasMedicines.com (PhRMA) connects patients to manufacturer programs but offers no clinical comparison data
- 40% of Americans have skipped or delayed a prescription due to financial concerns (Model N survey)
- HCPs report difficulty keeping track of which DTP programs exist and which patients qualify
- No existing resource combines eligibility checking, clinical drug comparison, and DTP program aggregation in one place — and critically, no existing resource does this from a position of true independence, with no pharma funding and no manufacturer bias

Target Audiences

Primary: Healthcare Providers (physicians, pharmacists, NPs, PAs) who need a quick reference to guide patients toward DTP options

Secondary: Patients — uninsured, underinsured, high-deductible plan holders, and those seeking medications not covered by insurance (e.g., weight loss GLP-1s)

Tertiary: Employer benefits teams, health plan consultants, and patient advocacy groups

2. Suggested Website Names

Your website name should communicate trust, simplicity, and its role as an independent guide. Here are options organized by strategy:

Strategy	Name Options	Notes
Descriptive / Direct	DirectRxGuide.com DTPRxFinder.com RxDirectAccess.com	Clear purpose, SEO-friendly, immediately tells visitors what the site does
Authority / Trust	RxCompareNow.com MyRxOptions.com RxNavigator.com	Positions the site as a trusted decision-making tool for patients and HCPs
Patient-Centered	SkipTheMiddleman.rx RxDirect.health ClearPathRx.com	Speaks to the patient pain point directly; memorable and shareable
Clinical / HCP-Focused	DTPFormulary.com RxAccessHub.com DirectCareRx.com	Appeals to healthcare professionals; positions site as a clinical resource

Recommendation: Secure 2–3 options and test with a small focus group of pharmacists and patients. Prioritize a .com domain for credibility. Consider also securing the .health or .rx variant.

3. Complete Sitemap

Below is the full site architecture organized by section. Each page is described with its purpose, key content elements, and priority level for launch.

3.1 Homepage

Element	Details	Priority
Hero Section	Headline: "Find the best DTP prescription drug program for you." Subheadline: Compare prices, check eligibility, and access medications directly from manufacturers.	P0 — Launch
Quick Search Bar	Search by drug name, condition, or manufacturer. Auto-suggest enabled.	P0 — Launch
Eligibility Checker CTA	Prominent button: "Check Your Eligibility" linking to the universal eligibility tool	P0 — Launch
Featured Programs	Rotating cards showing 4–6 popular DTP programs (e.g., LillyDirect, PfizerForAll, NovoCare) with key stats	P0 — Launch
Latest News Ticker	3–5 recent DTP headlines (new programs, price changes, regulatory updates)	P1 — Month 2
For HCPs Badge	Clear navigation path: "Are you a healthcare provider? Access our clinical tools."	P0 — Launch
Trust Indicators	"Independent. Unbiased. No pharma funding." — Badges, disclaimers, about link	P0 — Launch

3.2 Drug Comparison Tool

This is the core differentiator. No existing platform offers side-by-side clinical comparison of DTP-available drugs.

Element	Details	Priority
Condition Selector	Dropdown: Obesity, Diabetes, Migraine, Psoriasis, RA, Cholesterol, Blood Thinners, etc.	P0 — Launch
Drug Comparison Grid	Side-by-side table comparing: Brand name, generic name, manufacturer, DTP platform, DTP cash price, dosage forms, administration route, frequency, key efficacy data, common side effects, black box warnings, contraindications	P0 — Launch
Filter Options	Filter by: price range, administration route (oral vs. injectable), frequency, insurance acceptance	P0 — Launch
Direct Link Buttons	Each drug card includes "Go to [Manufacturer] DTP Site" button with outbound tracking	P0 — Launch

Savings Calculator	Input: current out-of-pocket cost. Output: potential savings via DTP vs. insurance vs. GoodRx	P1 — Month 2
PDF Export	"Download Comparison" button for HCPs to print and share with patients	P1 — Month 2

3.3 Universal Eligibility Checker

The eligibility checker is your highest-value tool. It converts visitors into engaged users and generates the data that powers monetization.

Step	User Input	Output
Step 1: Condition	Select your condition from a list (or search)	Narrows the drug options to relevant therapeutic area
Step 2: Insurance Status	Uninsured / Underinsured / High-deductible / Medicare / Medicaid / Employer-sponsored / Other	Flags which DTP programs may offer better pricing than insurance
Step 3: Current Medications	List current medications (optional)	Identifies potential interactions and therapeutic alternatives
Step 4: State of Residence	Select state	Filters by state-level DTP pharmacy licensing and availability
Step 5: Budget	Monthly budget range for medications	Ranks DTP options by affordability within patient's range
Results Page	N/A	Personalized list of eligible DTP programs ranked by price, with comparison data and direct links to manufacturer sites

3.4 Education Hub

3.4a Patient Education

Article / Page	Description	Priority
What Is DTP Prescribing?	Plain-language explainer: how DTP works, who it benefits, and what to watch out for	P0 — Launch
DTP vs. Traditional Pharmacy	Comparison of the two models: cost, convenience, safety, insurance implications	P0 — Launch
Is DTP Right for Me?	Decision tree / quiz helping patients determine if DTP makes sense for their situation	P0 — Launch
Understanding DTP Pricing	How cash-pay pricing works, why it may or may not beat insurance, hidden costs	P0 — Launch

Safety Considerations	Drug interactions, importance of sharing DTP purchases with your PCP, medication reconciliation	P0 — Launch
GLP-1 Buyer's Guide	Specific guide for the highest-demand DTP category: Zepbound vs. Wegovy vs. alternatives	P0 — Launch
Medicare & DTP	How Medicare patients can (and cannot) use DTP programs, IRA pricing implications	P1 — Month 2
Glossary of Terms	PBM, MFN, formulary, prior authorization, step therapy, biosimilar, etc.	P1 — Month 2

3.4b HCP Education

Article / Page	Description	Priority
DTP Clinical Reference Guide	Comprehensive overview for HCPs: which drugs are available via DTP, clinical considerations, how to counsel patients	P0 — Launch
Medication Reconciliation & DTP	How to account for DTP-purchased medications in patient records, EHR documentation tips	P0 — Launch
Regulatory Landscape	Anti-kickback statute considerations, OIG guidance, state licensing, FDA oversight	P1 — Month 2
Talking to Patients About DTP	Communication scripts and frameworks for discussing DTP options during appointments	P0 — Launch
CE/CME Content	Continuing education modules on DTP prescribing (future partnership opportunity)	P2 — Month 4+
Formulary Impact Analysis	How DTP programs interact with health plan formularies and prior authorization	P1 — Month 2

3.5 DTP Program Directory

Individual profile pages for every active DTP program. This is your SEO engine — each page targets long-tail searches like "LillyDirect eligibility" or "Pfizer DTP price for Eliquis."

Program	Manufacturer	Key Drugs	Launch Status
LillyDirect	Eli Lilly	Zepbound, Mounjaro, Emgality, Jardiance	Active since Jan 2024
PfizerForAll	Pfizer	Nurtec, Paxlovid, Eliquis (with BMS), vaccines	Active since Aug 2024
NovoCare Pharmacy	Novo Nordisk	Wegovy, Ozempic	Active since Mar 2025
AmgenNow	Amgen	Repatha (cholesterol)	Active since Oct 2025
Eliquis 360 Support	BMS / Pfizer	Eliquis (blood thinner)	Active since Sep 2025
BMS Patient Connect	Bristol Myers Squibb	Sotyktu (psoriasis)	Active since Jan 2026
Novartis DTP	Novartis	Cosentyx (psoriasis, PsA, AS)	Active since Nov 2025
Boehringer DTP	Boehringer Ingelheim	Various (details emerging)	Active since late 2025
Genentech DTP	Genentech/Roche	Various (details emerging)	Active since late 2025
AstraZeneca DTP	AstraZeneca	Farxiga (diabetes), others via TrumpRx	Active since Oct 2025
TrumpRx.gov	Federal Government	43 medications from multiple manufacturers	Active since Feb 2026
AmericasMedicines.com	PhRMA	Multi-manufacturer directory	Active since Jan 2026

Each program profile page should include: overview and history, eligible conditions, available medications with pricing, how to enroll (step-by-step), eligibility requirements, telehealth partner information, shipping/delivery details, insurance acceptance policy, patient reviews/ratings (future), and a direct link to the manufacturer's DTP site.

4. Content Calendar & Launch Phases

Phase 1: MVP Launch (Weeks 1–6)

Week	Deliverable	Details
Week 1–2	Core site structure	Homepage, navigation, branding, hosting setup (Vercel/Netlify)
Week 2–3	Drug comparison tool (v1)	Start with 3 therapeutic areas: Obesity/GLP-1, Migraine, Diabetes
Week 3–4	Eligibility checker (v1)	Basic 5-step flow with top 10 DTP programs
Week 4–5	Education content (v1)	6 core patient articles + 3 HCP articles
Week 5–6	Program directory (v1)	Profile pages for top 8 DTP programs
Week 6	Launch	Soft launch, social media announcement, outreach to pharmacy communities

Phase 2: Growth (Months 2–4)

- Expand drug comparison to 6+ therapeutic areas (add: psoriasis, RA, cholesterol, blood thinners)
- Add savings calculator to comparison tool
- Publish Medicare-specific and employer-specific content
- Launch email newsletter: "DTP Rx Weekly" covering new programs, price changes, regulatory updates
- Begin SEO optimization on program directory pages
- Add PDF export for HCP drug comparisons
- Outreach to pharmacy schools, medical associations, patient advocacy groups

Phase 3: Monetization (Months 4–8)

- Implement affiliate tracking for outbound links to manufacturer DTP sites
- Launch sponsored content program (clearly labeled, editorial independence maintained)
- Develop premium HCP toolkit (downloadable formulary guides, patient handouts)
- Explore CE/CME partnership for accredited DTP education modules
- Pitch to employer benefits consultants as a resource for their clients
- Explore B2B licensing of drug comparison data to EHR platforms or health plans

5. Monetization Strategy

Multiple revenue streams protect against dependency on any single source. Below is a prioritized monetization roadmap:

Revenue Stream	Description	Timeline	Potential
Affiliate / Referral Fees	Earn per-click or per-enrollment fees when users click through to manufacturer DTP sites. Track with UTM parameters.	Month 4+	High
Display Advertising	Programmatic ads from healthcare-adjacent advertisers (telehealth, wellness, insurance). Use Google AdSense or Mediavine.	Month 3+	Medium
Sponsored Content	Manufacturer-sponsored educational content clearly labeled as such. Must not compromise editorial independence.	Month 6+	High
Premium HCP Tools	Subscription or one-time purchase for downloadable clinical comparison guides, patient counseling sheets, formulary impact reports.	Month 6+	Medium
Newsletter Sponsorship	Sponsored placement in the weekly "DTP Rx Weekly" email newsletter.	Month 4+	Medium
B2B Data Licensing	License aggregated (non-PII) comparison data and market insights to health plans, PBMs, employer benefits consultants.	Month 8+	High
CE/CME Partnerships	Partner with accredited CE providers to offer DTP-focused continuing education. Revenue share model.	Month 8+	Medium
Consulting / Speaking	Leverage your PharmD expertise and platform authority for consulting engagements and conference speaking.	Month 6+	Medium

Important: Maintain editorial independence and transparency. Clearly disclose any financial relationships with manufacturers. Your credibility as an independent resource is your most valuable asset.

6. SEO & Content Strategy

Target Keywords

Keyword Category	Example Keywords	Search Intent	Content Type
Brand + DTP	"LillyDirect eligibility" "PfizerForAll price" "NovoCare Wegovy cost"	Navigational / Transactional	Program profile pages
Drug + DTP	"buy Zepbound without insurance" "Eliquis direct from manufacturer" "Wegovy cash price"	Transactional	Drug comparison pages
Educational	"what is DTP prescribing" "direct to patient pharmacy" "DTP vs pharmacy"	Informational	Education hub articles
Condition + Cost	"cheapest GLP-1 for weight loss" "affordable migraine medication" "psoriasis drug without insurance"	Transactional	Comparison tool + articles
HCP-Focused	"DTP prescribing guide for pharmacists" "medication reconciliation DTP" "counsel patients DTP drugs"	Informational	HCP education section
Competitor / Comparison	"TrumpRx vs LillyDirect" "GoodRx vs DTP pharmacy" "Cost Plus Drugs vs manufacturer direct"	Commercial Investigation	Comparison articles

Content Velocity

- Launch: 20–25 pages of core content (articles, tools, program profiles)
- Months 2–4: 3–4 new articles per week (news updates, new program profiles, comparison updates)
- Months 4+: 2–3 articles per week + weekly newsletter + monthly HCP report
- Evergreen content: Update drug comparison data and program profiles at least bi-weekly

Distribution Channels

- LinkedIn: Primary channel for HCP audience. Share clinical insights, new program announcements, opinion pieces on DTP policy
- X/Twitter: Real-time DTP news, regulatory updates, price change alerts

- Pharmacy communities: Reddit r/pharmacy, pharmacy Facebook groups, ASHP/APhA forums
- Email newsletter: Weekly digest with new programs, price changes, and clinical updates
- Podcast (future): "The DTP Rx Report" — weekly 15-minute updates on the DTP landscape

7. Competitive Landscape & Positioning

Competitor	What They Do	What They DON'T Do	Your Advantage
TrumpRx.gov	Government portal aggregating DTP programs with MFN pricing	No clinical comparison, no eligibility tool, limited drug selection (43 drugs), no HCP resources	Clinical depth, eligibility checker, broader coverage, HCP tools
AmericasMedicines.com	PhRMA directory connecting patients to manufacturer DTP programs	No drug comparison, no eligibility screening, industry-funded (bias concerns), no clinical data	Independence, clinical comparison, eligibility tool, unbiased positioning
GoodRx	Prescription price comparison and coupons	Not DTP-specific, doesn't aggregate manufacturer programs, no clinical comparison, insurance/PBM-adjacent	DTP specialization, manufacturer program aggregation, clinical context
Cost Plus Drugs	Mark Cuban's cash-pay generic pharmacy	Focused on generics, not brand-name DTP, no comparison tools, no HCP resources	Brand-name DTP focus, clinical comparison, manufacturer program directory
Drugs.com	Drug information, price comparison, interaction checker	Not DTP-focused, no eligibility tool, no manufacturer program directory, general-purpose	DTP specialization, eligibility checker, program aggregation
Individual Manufacturer Sites	LillyDirect, PfizerForAll, NovoCare, etc.	Only show their own drugs, no cross-manufacturer comparison, no independent clinical analysis	Cross-manufacturer comparison, unbiased, one-stop resource

Your positioning statement: "The only independent, pharmacist-built platform that cuts through pharma bias to help patients and providers discover, compare, and access every direct-to-patient prescription drug program in the United States — because patients deserve a neutral guide, not a manufacturer storefront."

8. Technical Recommendations

Since you have no coding background, here are practical, no-code and low-code options for building this site:

Platform Options

Platform	Best For	Cost	Coding Needed?
Webflow	Beautiful, responsive sites with CMS for drug data. Best balance of power and ease.	$29–$49/mo	No
WordPress + Elementor	Content-heavy sites, strong SEO plugins, huge ecosystem. Most pharmacy/health sites use WP.	$10–$30/mo hosting	Minimal
Bubble.io	Complex interactive tools (eligibility checker). Most powerful no-code option for app-like features.	$29–$119/mo	No (but learning curve)
Framer	Modern, fast sites with CMS. Good for a sleek, startup-style look.	$15–$30/mo	No
Claude + Vercel	Claude builds your pages as code, you deploy to Vercel for free. Most flexible, zero cost to start.	Free	No (Claude writes the code)

Recommended Stack

Start: Claude builds your site components (React/HTML) + deploy to Vercel (free). This gets you launched with zero cost and maximum flexibility.

Scale: Migrate to WordPress or Webflow when you need a CMS to manage 50+ drug listings and program profiles without touching code.

Tools: Airtable or Google Sheets as your drug database (Claude can build tools that pull from these). Mailchimp or Beehiiv for newsletter. Google Analytics for tracking.

9. Legal & Compliance Considerations

This section outlines key legal areas to address. Consult a healthcare attorney before launch.

- FDA Compliance: You are not prescribing or selling drugs. You are an information aggregator. However, ensure all drug information is sourced from FDA-approved prescribing information and clearly attributed.
- Anti-Kickback Statute: If you earn referral fees from manufacturer DTP sites, consult legal counsel on AKS implications, especially for patients with federal healthcare program coverage (Medicare/Medicaid).
- FTC Disclosure: Any affiliate relationships, sponsored content, or financial ties to manufacturers must be clearly disclosed per FTC guidelines.
- HIPAA: If your eligibility checker collects health information, determine whether you are a covered entity or business associate. Anonymized, non-stored data may avoid HIPAA scope, but get legal confirmation.
- State Pharmacy Laws: You are not operating as a pharmacy, but state regulations vary. Ensure your site clearly states it is an informational resource, not a healthcare provider.
- Medical Disclaimer: Include prominent disclaimers that the site does not provide medical advice, does not replace the patient-provider relationship, and that all treatment decisions should be made with a healthcare provider.
- Privacy Policy: Implement a clear privacy policy, especially if using analytics, cookies, or collecting any user data through the eligibility checker.
- Terms of Use: Include terms governing site use, limitation of liability, and accuracy disclaimers for drug pricing (which changes frequently).

10. Success Metrics & KPIs

Metric	Month 1 Target	Month 3 Target	Month 6 Target
Monthly unique visitors	500–1,000	5,000–10,000	25,000–50,000
Eligibility checker completions	50–100	500–1,000	2,500–5,000
Drug comparison tool uses	100–200	1,000–2,000	5,000–10,000
Outbound clicks to DTP sites	50–100	500–1,000	2,500–5,000
Newsletter subscribers	100	1,000	5,000
LinkedIn followers	200	1,000	3,000
Avg. time on site	> 2 min	> 3 min	> 3 min
Domain authority (Moz/Ahrefs)	5–10	15–20	25–35

11. Immediate Next Steps

1. **Secure your domain name.** Register your top 2–3 preferred names immediately. Domains in the health/Rx space are being claimed rapidly.
2. **Build a drug database.** Start with a Google Sheet tracking all DTP programs, drugs, prices, eligibility criteria, and links. Claude can help you structure this.
3. **Write your first 3 articles.** "What Is DTP Prescribing?", "Is DTP Right for Me?", and "GLP-1 Buyer's Guide" will drive early traffic.
4. **Ask Claude to build your eligibility checker.** This is your MVP feature. Claude can create a working interactive prototype in a single conversation.
5. **Consult a healthcare attorney.** Before monetizing, get legal guidance on affiliate fees, AKS compliance, and HIPAA scope.
6. **Establish your online presence.** Create a LinkedIn page and begin posting about DTP trends. Build authority before launch.

12. Data Collection Architecture

The eligibility checker is the platform's highest-value tool and its most important source of proprietary data. Because no personally identifiable information (PII) is collected — no names, dates of birth, contact information, or insurance ID numbers — the data gathered is anonymous behavioral data. This data is valuable for content strategy, partnership negotiations, B2B licensing, and platform improvement. Three layers of data collection will be implemented from launch day.

12.1 Layer 1: Analytics Event Tracking (Google Analytics 4)

Google Analytics 4 (GA4) will be integrated into the eligibility checker at no cost. Every user interaction fires a custom event, enabling aggregate reporting on platform usage patterns. GA4 event tracking will capture the following data points: condition category selected (e.g., GLP-1/Obesity, Migraine, Diabetes), insurance status input (Uninsured, High-Deductible, Employer-Sponsored, etc.), step completion and drop-off rates across the 5-step checker flow, drug outputs displayed on the results page, DTP program links clicked through from results, and state of residence selected. This data is aggregate and cannot be tied to any individual user. It informs content prioritization, SEO strategy, and platform optimization.

12.2 Layer 2: Anonymous Session Database (Supabase)

A lightweight anonymous session log will be maintained in Supabase (free tier at launch). Each completed eligibility checker session generates a single database row containing a randomly generated session ID (no user identifiers), condition selected, insurance status, state of residence, monthly budget range, top drug output, and DTP program link clicked. No IP addresses, device identifiers, or any information that could re-identify the user will be stored. This dataset enables granular querying — for example, identifying the most common condition-

insurance-state combinations, or tracking which DTP programs receive the most referrals by therapeutic area. This data is the foundation for the B2B data licensing revenue stream planned for Month 8+.

Sample Anonymous Session Row:

```
Session ID: a7x92k | Condition: GLP-1/Obesity | Insurance: High-Deductible |
State: TX | Budget: $200-400 | Drug Output: Zepbound | DTP Clicked: LillyDirect

Session ID: b3m15p | Condition: Migraine | Insurance: Uninsured | State: FL |
Budget: $100-200 | Drug Output: Nurtec | DTP Clicked: PfizerForAll
```

12.3 Layer 3: UTM Tracking on Outbound Links

Every outbound link to a manufacturer DTP site (LillyDirect, NovoCare, PfizerForAll, TrumpRx, etc.) will include UTM parameters that identify the referral source, medium, and campaign. This enables precise attribution of referral traffic to each manufacturer partner. UTM tracking is the mechanism that supports future affiliate revenue arrangements and allows the platform to demonstrate its referral value to manufacturers and sponsors with concrete data. Example UTM-tagged link: lillydirect.com/?utm_source=directrxguide&utm_medium=eligibility_checker&utm_campaign=glp1_obesity.

12.4 Re-Identification Risk Management

Although no PII is collected, the combination of condition, state, insurance status, and budget range could theoretically approach re-identifiability for rare conditions in small states. To mitigate this risk, the following practices will be followed: session data for rare conditions (fewer than 50 monthly sessions) will be reported only at aggregate level and not stored as individual rows; state-level data will be grouped into regions for any public-facing reporting; no combination of more than three data fields will be published in any external report without confirming minimum cell counts of 50 or more.

12.5 Strategic Value of Data Asset

The anonymous behavioral dataset generated by the eligibility checker represents a proprietary market intelligence asset with multiple monetization pathways. For content strategy, dominant condition-insurance combinations inform article prioritization and SEO investment. For the HCP newsletter, aggregate platform insights (“most searched condition this month”) provide original data unavailable elsewhere. For manufacturer partnerships, referral attribution data demonstrates platform value and supports sponsored content pricing. For B2B licensing, aggregated condition-insurance-state demand patterns are valuable to health plans, PBMs, and employer benefits consultants evaluating DTP program coverage decisions. This data asset grows in value with every user interaction and is a meaningful long-term competitive advantage.

13. Privacy Statement & Policy Framework

A clear, accessible privacy policy is a legal requirement and a trust-building asset. Given the health-adjacent nature of the platform, transparency about data practices is especially important. The privacy policy must be linked prominently in the site footer and referenced explicitly on the eligibility checker before users begin the tool. The following framework outlines

the core privacy policy components required at launch. Final policy language should be reviewed by a healthcare attorney before publication.

13.1 What We Collect

The platform collects two categories of data. Anonymous usage data includes eligibility checker inputs (condition, insurance status, state, budget range) and outputs (drug recommendations, DTP program links clicked). This data is collected without any name, contact information, date of birth, or other identifying information and cannot be traced back to any individual. Standard technical data includes browser type, device type, pages visited, and time on site, collected automatically by Google Analytics 4. The platform does not collect names, email addresses, phone numbers, dates of birth, insurance ID numbers, prescription details, or any other personally identifiable information through the eligibility checker or any other site tool unless a user explicitly submits their email address to subscribe to the newsletter, in which case only the email address is collected and stored separately from all usage data.

13.2 What We Do Not Collect

The platform explicitly does not collect the following: full name, date of birth, Social Security number, insurance member ID or group number, prescription drug history, diagnosis or medical records, income verification or financial account information, IP addresses linked to health condition inputs, or any biometric data. The eligibility checker is designed to run entirely without user accounts or login requirements. Users are never required to identify themselves to use any feature of the platform.

13.3 How We Use Data

Anonymous usage data is used for the following purposes: improving platform tools and content based on user behavior patterns; publishing aggregate market insights in the DTP Rx Weekly newsletter and partner reports; demonstrating referral value to manufacturer and sponsor partners; and informing B2B data licensing products offered to health plans, PBMs, and employer benefits consultants. All data shared externally is aggregated and contains no information that could identify any individual user. Newsletter email addresses are used solely to send the DTP Rx Weekly newsletter and are never sold, shared with manufacturers, or combined with usage data.

13.4 HIPAA Position

The platform is designed to operate outside HIPAA's scope as a general health information website and does not function as a covered entity or business associate. No protected health information (PHI) as defined under HIPAA is collected, stored, or transmitted. The eligibility checker collects condition categories and insurance status inputs anonymously without tying them to an identifiable individual, which does not constitute PHI under the HIPAA Privacy Rule. This position should be confirmed with a healthcare attorney prior to launch. If future features such as user accounts, saved results, or provider-facing tools are introduced, HIPAA scope should be re-evaluated at that time.

13.5 Cookies & Tracking Technologies

The platform uses cookies and similar tracking technologies for the following purposes: analytics (Google Analytics 4 to understand aggregate site usage), functional cookies (to remember user preferences within a single session), and UTM parameters on outbound links to

track referral traffic to manufacturer DTP sites. The platform does not use advertising cookies, behavioral targeting cookies, or third-party tracking pixels from pharmaceutical manufacturers or advertisers. A cookie consent banner will be displayed to users on first visit in compliance with applicable state privacy laws. Users may opt out of analytics tracking via the banner without affecting their ability to use any platform feature.

13.6 Third-Party Links & Outbound Destinations

The platform links to third-party manufacturer DTP websites including but not limited to LillyDirect, NovoCare, PfizerForAll, AstraZeneca Direct, TrumpRx.gov, and others. Once a user clicks through to a third-party site, that site's own privacy policy governs data collection. The platform is not responsible for the privacy practices of any linked third-party sites and recommends that users review the privacy policy of any manufacturer DTP site before submitting personal or health information. All outbound links are clearly labeled as external destinations.

13.7 User Rights & Contact

Newsletter subscribers may unsubscribe at any time via the unsubscribe link included in every email. Because the eligibility checker collects no identifying information, there is no individual usage data to access, correct, or delete. Users in states with applicable privacy laws (including California CCPA and Virginia VCDPA) may submit privacy requests via the contact email published on the site. The platform will respond to all privacy inquiries within 30 days. The privacy policy will be reviewed and updated at least annually and any time a material change to data practices is made. The date of the most recent update will be displayed prominently at the top of the privacy policy page.

13.8 Eligibility Checker Privacy Notice (In-Tool)

The following notice will be displayed prominently at the start of the eligibility checker, before Step 1:

> **Your Privacy is Protected**
>
> This tool does not collect your name, contact information, insurance ID, or any information that identifies you personally. Your answers are used only to generate your personalized DTP program results and are stored anonymously to help us improve this tool. We are not a healthcare provider and this tool does not constitute medical advice. Always consult your healthcare provider before changing how you obtain or pay for your medications.

14. SEO & LLM Discoverability Strategy

The platform operates in two distinct discovery environments that require separate but complementary strategies. Traditional search engine optimization (SEO) drives discovery through Google, Bing, and similar search engines via keyword ranking. LLM discoverability drives citation and recommendation by AI assistants such as ChatGPT, Claude, Perplexity, and Gemini when users ask health and drug pricing questions. Most health content platforms optimize only for traditional SEO. Building for LLM discoverability from day one is a meaningful competitive advantage that very few platforms in this space are pursuing deliberately. This

section covers both strategies and the content architecture required to execute them simultaneously.

14.1 Traditional SEO Foundation

The platform's SEO strategy is built around three keyword tiers that map directly to user intent and content type. Each tier requires a distinct content format and internal linking approach.

Tier 1 — Navigational / Brand Keywords. Users searching for a specific manufacturer program by name. Examples: "LillyDirect eligibility", "NovoCare Wegovy cost", "PfizerForAll how to enroll", "TrumpRx drug list". Target content: individual DTP program profile pages in the program directory. Each profile page should be the most comprehensive independent resource on that specific program available anywhere on the internet. These pages will rank for navigational queries and capture users already in research mode for a specific program.

Tier 2 — Transactional Keywords. Users ready to act on drug access. Examples: "buy Zepbound without insurance", "cheapest GLP-1 for weight loss", "Wegovy cash price 2026", "Gonal-F direct from manufacturer". Target content: drug comparison pages and eligibility checker entry points. These keywords have the highest commercial intent and drive the most valuable traffic — users who are ready to access a DTP program today.

Tier 3 — Informational Keywords. Users researching and learning. Examples: "what is direct to patient prescribing", "how does DTP pharmacy work", "is TrumpRx legit", "DTP vs GoodRx". Target content: education hub articles and glossary pages. These keywords build domain authority, drive newsletter signups, and establish the platform as the go-to independent reference for DTP education. They also feed heavily into LLM citation potential, which is addressed in detail in Section 14.3.

Technical SEO requirements at launch. Every page must include a unique title tag and meta description optimized for its primary keyword. All images require descriptive alt text. Page load speed must be under 2.5 seconds on mobile (Cloudflare CDN handles this). Every drug profile and article must implement schema markup as described in Section 14.2. An XML sitemap must be submitted to Google Search Console at launch. Internal linking between related drug pages, condition pages, and education articles must be deliberate and consistent — every drug page should link to its relevant condition comparison page, which links to the eligibility checker, which links back to drug pages. This creates the interlinking structure that signals topical authority to Google.

14.2 Structured Data & Schema Markup

Schema markup is machine-readable metadata embedded in page code that tells both search engines and LLMs precisely what a page is about and what facts it contains. It is one of the most important technical investments for both traditional SEO and LLM discoverability and must be implemented from launch day on every content type.

Drug profile pages: Drug schema. Each DTP program profile and drug comparison entry must implement Drug schema (schema.org/Drug) specifying drug name, brand name, manufacturer, non-proprietary name, drug class, available strength, administration route, and DTP cash price. This enables search engines to surface drug facts directly in results and signals to LLMs that this page is an authoritative structured source for drug information.

Education articles: Article and MedicalWebPage schema. All patient and HCP education articles must implement MedicalWebPage schema specifying the author's credentials (PharmD), medical specialty, last reviewed date, and audience type (patient or HCP). This is the

primary schema type that signals medical credibility to both Google's E-E-A-T (Experience, Expertise, Authoritativeness, Trustworthiness) system and to LLMs evaluating whether a source is worth citing.

FAQ sections: FAQPage schema. Every article and drug profile must include a FAQ section at the bottom with 4–6 questions and answers marked up with FAQPage schema. FAQ schema drives Google's People Also Ask boxes, which are high-visibility placements that drive significant traffic. More importantly, FAQ sections written as direct question-and-answer pairs are the format most likely to be extracted and cited verbatim by LLMs responding to user questions.

Organization and author schema: E-E-A-T signals. Site-wide Organization schema must identify the platform as an independent health information resource. Author schema on every article must reference the pharmacist author's credentials, professional affiliations, and a link to an author bio page. The author bio page itself should link to verifiable external credentials. This establishes the E-E-A-T signals that Google's Quality Rater Guidelines require for health content to rank well and that LLMs use to evaluate source credibility.

14.3 LLM Discoverability Strategy

When a patient or HCP asks ChatGPT, Claude, Perplexity, or Gemini a question like "what is the cheapest way to get Wegovy without insurance" or "what DTP programs exist for GLP-1 medications", the LLM draws on content it has indexed or can retrieve via web search. Being cited by LLMs is a rapidly growing traffic channel that most health content platforms are not yet optimizing for deliberately. The following content architecture principles maximize LLM citation probability.

14.3a Atomic Statement Writing

An atomic statement is a single, self-contained sentence that expresses exactly one verifiable fact and can be understood completely without surrounding context. LLMs are significantly more likely to extract and cite atomic statements than dense paragraphs because they can be lifted verbatim into a response without distortion. Every piece of content published on the platform must be written with atomic statements as the foundational unit of information delivery.

The atomic statement principle in practice. Instead of writing "Zepbound is a GLP-1 medication made by Eli Lilly that can be purchased directly through LillyDirect for weight loss patients who want to pay cash and avoid using their insurance, with prices starting around $299 per month depending on dose", the content is broken into discrete atomic statements: "Zepbound (tirzepatide) is a GLP-1 receptor agonist manufactured by Eli Lilly." "Zepbound is FDA-approved for chronic weight management in adults with obesity or overweight with a weight-related condition." "Zepbound is available for direct purchase through LillyDirect, Eli Lilly's direct-to-patient platform." "The cash-pay price for Zepbound through LillyDirect starts at $299 per month for the 2.5mg dose as of March 2026." "Purchasing Zepbound through LillyDirect requires a valid prescription and does not accept insurance at point of sale." Each statement is independently citable, verifiable, and extractable by an LLM without requiring surrounding context.

Content writing standard for atomic statements. Every article must open with a summary block of 4–6 atomic statements that capture the most important facts on the page. These summary blocks are the highest-probability LLM citation targets on the entire platform. Each atomic statement must name the drug or program explicitly (not "it" or "this medication"), include a specific numerical fact where applicable (price, discount percentage, dose), state the source

or basis of the fact where verifiable, and be accurate as of the stated date with a clearly marked last-updated timestamp.

14.3b Question-Answer Content Pairs

LLMs are trained on and optimized to respond to questions. Content structured as explicit question-and-answer pairs is significantly more likely to be retrieved and cited when an LLM is answering a related user query. Every article, drug profile, and program directory page must include a dedicated FAQ section structured as explicit Q&A pairs where the question is written exactly as a user would type it into an LLM or search engine. Examples of well-formed LLM-optimized questions: “Can I buy Wegovy directly from Novo Nordisk without insurance?” “What is the cash price for Zepbound through LillyDirect in 2026?” “Does TrumpRx cover fertility drugs?” “What is the difference between TrumpRx and LillyDirect?” “Can I use a DTP program if I have employer-sponsored insurance?” Each answer must be written as a concise atomic statement or a short paragraph of 2–3 atomic statements. Answers must never exceed 100 words. Verbose answers are less likely to be cited verbatim by LLMs.

14.3c Comparison and Definitional Content

LLMs are frequently asked comparison questions and definitional questions. Content that directly answers these question types in a structured format has the highest citation probability of any content format. The platform must prioritize two content types specifically for LLM discoverability. Definitional pages are standalone pages that define a single concept clearly and completely. Examples: “What is a Direct-to-Patient Pharmacy Program?”, “What is Most Favored Nation Drug Pricing?”, “What is a Pharmacy Benefit Manager?”. Each definitional page must open with a single atomic definitional statement that can stand alone as a complete answer. Comparison pages directly contrast two or more options on a set of defined dimensions. Examples: “TrumpRx vs LillyDirect: What Is the Difference?”, “GoodRx vs DTP Programs: Which Saves More?”, “Zepbound vs Wegovy: DTP Pricing and Access Compared”. Comparison pages must include a structured comparison table and a summary section of atomic statements that captures the key differences. These pages are extremely high-value for LLM citation because they provide the synthesized answer an LLM needs to respond to a comparison query.

14.3d Transparent Sourcing and Timestamp Discipline

LLMs weight content from sources that demonstrate factual transparency and recency. Every piece of content must include a visible “Last Updated” date at the top of the page. Drug pricing data must be sourced and dated explicitly within the content, for example: “As of March 2026, per TrumpRx.gov.” Any statistic or claim must link to its primary source. The DTP landscape changes rapidly — a page with an outdated price that contradicts the current manufacturer price will be deprioritized by LLMs in favor of more current sources. A bi-weekly content review process must be established to update drug prices and program details on all program profile pages. Pages that have not been reviewed in more than 30 days must display a “Pending Review” flag until updated. Freshness is both an SEO signal and an LLM credibility signal.

14.3e llms.txt File

An emerging standard for LLM discoverability is the llms.txt file, analogous to robots.txt for traditional search crawlers. This is a plain-text file placed at the root of the domain (directrxguide.com/llms.txt) that tells LLM crawlers which pages are the most authoritative and citable on the site, what the site’s primary purpose and scope is, and which content is most current and reliable. While not yet universally adopted by all LLMs, Perplexity, Claude, and others are beginning to respect llms.txt as a discoverability signal. The file should be created at

launch and maintained as the platform grows. It should list the platform's top definitional pages, comparison pages, and drug profile pages as priority citation targets.

14.3f Wikipedia and Wikidata Presence

Most major LLMs were trained heavily on Wikipedia and continue to use it as a high-trust source. Contributing accurate, well-sourced information to relevant Wikipedia articles — such as the articles on direct-to-consumer pharmaceutical advertising, pharmacy benefit managers, GLP-1 receptor agonists, and individual drug articles — and ensuring the platform is cited as a reference where appropriate is a meaningful LLM discoverability strategy. Similarly, ensuring that major DTP programs have accurate Wikidata entries with links to the platform's program profiles increases the probability of LLM citation. This activity is appropriate from Month 3 onwards once the platform has sufficient content quality to merit citation.

14.4 Content Template: Standard Article Structure

Every article and drug profile published on the platform must follow this standard structure to maximize both SEO performance and LLM citation probability. Deviating from this structure requires explicit justification.

1. Page Header — H1 title (keyword-optimized), author name and credentials (PharmD), published date, last updated date, and medical disclaimer link.

2. Key Facts Summary Block — 4–6 atomic statements summarizing the most important facts on the page. This is the highest-priority LLM citation target on every page. Must be visible above the fold.

3. Introduction Paragraph — 2–3 sentences framing the topic and its relevance to the patient or HCP reader. Written in plain language. No jargon without definition.

4. Main Content Body — Organized under H2 and H3 subheadings. Each section opens with an atomic statement that summarizes the section's key point. Paragraphs are short (3–4 sentences maximum). One idea per paragraph.

5. Comparison Table (where applicable) — Structured data table comparing options on defined dimensions. Tables are high-value for both featured snippet capture in Google and LLM data extraction.

6. FAQ Section — 4–6 question-and-answer pairs marked up with FAQPage schema. Questions written as users would phrase them to an LLM or search engine. Answers written as atomic statements of 1–3 sentences maximum. This section is the second-highest LLM citation target on every page after the Key Facts Summary Block.

7. Sources and Last Updated — Linked primary sources for all pricing data and clinical claims. Explicit last-updated date. Review schedule notation ("This page is reviewed bi-weekly. Next review due: [date].")

8. Related Content Links — Internal links to related drug pages, condition comparison pages, program directory profiles, and eligibility checker entry point. Minimum 3 internal links per page.

14.5 LLM Monitoring & Citation Tracking

As the platform grows, tracking LLM citation frequency becomes a meaningful performance metric. On a monthly basis, query each major LLM (ChatGPT, Claude, Perplexity, Gemini) with the platform's top target questions — such as "what is the cheapest way to get Wegovy without insurance" or "what DTP programs exist for GLP-1 medications" — and record whether the

platform is cited, what content is cited, and how the platform is characterized. This manual monitoring process takes approximately one hour per month and provides directional data on LLM discoverability progress. Tools such as Perplexity's citation tracking and emerging LLM SEO platforms (BrandMentions AI, Otterly.ai) can supplement manual monitoring as the category matures. Citation frequency from LLMs is a leading indicator of platform authority and should be tracked alongside traditional SEO metrics such as domain authority, organic traffic, and keyword rankings.

This document was prepared as a strategic planning guide. All drug program information is based on publicly available data as of March 2026 and should be verified before publication. Market conditions in the DTP space are evolving rapidly.